Supplementary Material

Supplementary Tables

Supplementary Table 1. Quality assessment of the included studies using the Newcastle-Ottawa Scale

Reference	Selection				Comparability	Outcome			Score
	A	B	C	D	E	F	G	H	
James, 2019 (24)	★	★	★	★	★	★		★	7
Goya, 2014 (17)	★	★	★	★	★★	★		★	8
Shibazaki, 2014 (25)	★	★	★	★		★		★	6
Park, 2015 (26)	★	★	★	★	★★	★		★	8
Li, 2017 (18)	★	★	★	★	★			★	6
Niu, 2017 (27)	★	★	★	★			★	★	6
Yang, 2018 (28)	★	★	★	★			★	★	6
Li, 2018 (29)	★	★	★	★			★	★	6
Gregorio, 2019 (19)	★	★	★	★	★★	★		★	8
Eldawoody, 2020 (30)	★	★	★	★	★	★		★	7

A, representativeness of the exposed cohort. B, selection of the non-exposed cohort. C, ascertainment of exposure. D, demonstration that outcome of interest was not present at start of study. E, comparability of cohorts based on the design or analysis. F, assessment of outcome. G, follow-up long enough for outcomes to occur. H, adequacy of follow-up of cohorts.

Supplementary Table 2. Results of conference abstracts: the association between BNP/NT-proBNP levels and prognosis of ICH

Reference	Country	No. of patients	Biomarker/ Blood collection time	Outcome measures	Follow-up	OR (95% CI), P-value	Statistical analysis	Cutoff value	BNP related to prognosis
Blanco, 2011 (1)	Spain	134	pro-BNP/ on admission	poor functional outcome: mRS > 3	3 months 6 months 1 year	4.718 (1.113-20.001) 7.215 (1.030-50.518) 9.389 (1.274-69.178)	–	450 pg/ml AUC: 0.770 sensitivity: 75.6% specificity: 70%	Yes
	Pro-BNP level: poor functional outcome > good outcome (3480±8736 vs 504±799 pg/ml; P = 0.013).								
Goya, 2013 (2)	Japan	304	BNP/ on admission	mortality	1 year	2.9 (1.457-5.931), P = 0.0026	MV	50 pg/ml –	Yes
	Median age: 72; male: 63%; baseline BNP level: 123.1±320.8 pg/ml.								
Prada, 2015 (3)	Spain	103	BNP/ on admission	mortality	–	1.1 (1.1-1.2), P = 0.02	MV	–	Yes
	Median age: 61.8; male: 66%; BNP level: death > survival (223.5±318.5 vs 82.4±95.6 pg/ml; P < 0.05).								
Radhakrish-nan, 2018 (4)	India	60	BNP/ –	mortality	in-hospital	–	MV	–	Yes
	Method of BNP detection: ELISA.								
Rotzel, 2019 (5)	Spain	120	BNP/ on admission	1. mortality 2. poor functional outcome: mRS > 2	–	–	UV	–	1. Yes 2. No
	Mean age: 63; male: 63%; BNP level: death > survival (199.1±37 vs 84.3±10 pg/ml; P = 0.005).								

BNP, brain natriuretic peptide; NT-proBNP, N-terminal pro-brain natriuretic peptide; ICH, intracerebral hemorrhage; OR, odds ratio; CI, confidence interval; mRS, modified Rankin Scale; AUC, area under the curve; MV, multivariate logistic regression; UV, univariate logistic regression; ELISA, enzyme-linked immunosorbent assay. BNP/NT-proBNP level was presented as mean ± SD.

References

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3. Aguillón Prada DL, Serrano Lázaro A, Huerta Bravo RC, Mesejo Arizmendi A, Segura Roca JM, Sanchis Miñana JC, et al. Brain injury biomarkers and inflammatory markers like pronostic factors on mortality in patients with spontaneous intracranial hemorrhage (medical-adults intensive care). *Intensive Care Medicine Experimental*. (2015) 3. doi: 10.1186/2197-425X-3-S1-A852
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